

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — What Real Accessibility and Inclusion Actually Look Like

Section 1.5 — Caregivers and the End of the Family Safety Net

Section 1.4 closed by naming the cliff at eighteen and the structural failure pattern it exposes. Section 1.5 examines what makes the cliff survivable for the population that does survive it, and what makes the cliff catastrophic for the population that does not. The answer, in both directions, is the same: family caregivers. The family is what the system is, in operation, depending on. The family is also what the system, in operation, is consuming.

This section documents three things. First, the scale of the caregiver burden the public system depends on, in dollars, hours, and human consequences. Second, what happens to the disabled adult when the family caregivers can no longer carry what the system has tacitly transferred to them — through illness, aging, or death. Third, the documented intergenerational pattern that the architecture's failures produce: poverty, trauma, and unsupported disability concentrating across generations of the same families, generating the next generation's disabled population the system will then under-support in the same way.

The lifecycle frame's fourth failure point is the moment the family safety net stops being available. The province has not built the architecture that the family has been substituting for. The architecture's absence becomes visible the moment the substitute fails. That is the failure Section 1.5 examines.

What the Family Is Currently Carrying

Approximately eight million Canadians provide unpaid care to family members or friends with long-term health conditions, physical or mental disabilities, or age-related needs. Twenty-two percent of Canadians (approximately 6.8 million people) are family caregivers in any given year. Half of all Canadians will be family caregivers at some point in their lives. The Canadian Centre for Caregiving Excellence documents that caregivers spend more than 5.7 billion hours per year providing unpaid care across the country.

Researchers at the University of Alberta and University of Manitoba estimated, in a 2022 study published in the *Journal of Family and Economic Issues*, that the replacement-cost value of this unpaid family care work falls between \$97.1 billion and \$112.7 billion annually. The lower figure is the most-cited number across Canadian caregiving research. Comparable Statistics Canada and Canadian Institute for Health Information analyses have produced consistent estimates. Family caregivers in Canada provide approximately 75 percent of all care services delivered at home. The Government of Canada's National Seniors Council references unpaid caregiving in the range of \$24 billion to \$31 billion annually for the in-home portion specifically.

The Organisation for Economic Co-operation and Development has estimated that the Canadian family care sector is more than ten times the size of the formal care workforce. The implication is structural. The

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formal care system that governments fund and report on is a small fraction of the actual care economy. The larger, unfunded, unrecognized portion is the family. The province's disability architecture — AISH, PDD, FSCD, the wraparound healthcare and housing systems — is a public-facing scaffold whose actual operation depends on a much larger private substrate of unpaid family labour. When the formal scaffold fails or is under-built, the private substrate absorbs the difference. When the private substrate fails, there is nothing left.

The cost to the family is documented across three domains, identified by Norah Keating, Janet Fast, Donna Lero and colleagues in a 2014 systematic review published in the *Journal of the Economics of Ageing*. The three domains are care labour, employment restrictions, and out-of-pocket expenses. Each is measurable. Each is borne by the caregiver. None is repaid by the system the caregiver is, in effect, subsidizing.

Care labour is the time itself: hours not available for paid employment, sleep, education, health management, or the caregiver's own care needs. *Employment restrictions* include job loss, absenteeism, reduced hours, foregone career advancement, and reduced lifetime earnings. The 2018 Statistics Canada General Social Survey on Caregiving and Care Receiving documented that caregivers with the lowest household incomes report care-related expenditures averaging more than 20 percent of their annual income. The 2024 Canadian Centre for Caregiving Excellence National Caregiving Survey documented that one in ten caregivers reported having to leave bills unpaid or pay them late, or having to draw on long-term savings, in order to meet caregiving expenses. *Out-of-pocket expenses* include the costs the formal system does not cover — equipment, medications not on formulary, transportation, home modifications, respite care, and the private assessments and services that fill the gaps the public system has not closed.

The cumulative effect is that family caregivers in Canada subsidize the formal care system at approximately \$97 billion per year, in a distribution that falls hardest on the families with the least capacity to absorb it. Living arrangement explained 81 to 83 percent of the variance in the value of unpaid care work in the 2022 Canadian study cited above. Caregivers who co-reside with the person they care for are doing the heaviest lifting and bearing the largest share of the unmeasured cost.

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WHAT THE FAMILY IS CURRENTLY CARRYING

8 million Canadians provide unpaid care to family members or friends with disabilities, chronic illness, or age-related needs. Half of all Canadians will be caregivers at some point in their lives. *Source: Canadian Centre for Caregiving Excellence; Statistics Canada.*

5.7 billion unpaid caregiving hours per year, valued at \$97.1 billion to \$112.7 billion annually in replacement cost. Family caregivers provide approximately 75 percent of all in-home care delivered in Canada. *Source: Journal of Family and Economic Issues, 2022 (UofA / UofManitoba); Statistics Canada 2018 GSS.*

The Canadian family care sector is more than ten times the size of the formal care workforce. *Source: OECD.*

One in ten caregivers reports having to leave bills unpaid or use long-term savings to meet caregiving expenses. Caregivers with the lowest household incomes report care-related expenditures averaging more than 20 percent of annual income. *Source: CCCE National Caregiving Survey 2024; Statistics Canada GSS 2018.*

Three documented cost domains: care labour, employment restrictions, and out-of-pocket expenses. None repaid by the system the family is subsidizing. *Source: Keating, Fast, Lero et al., Journal of the Economics of Ageing, 2014.*

The Health Cost to Caregivers

The economic cost of caregiving is the visible cost. The health cost to caregivers themselves is the cost the system does not register at all, until the caregiver becomes a patient and the cost moves to a different line item in the same provincial budget.

The most heavily cited longitudinal study of caregiver mortality, the *Caregiver Health Effects Study* published in JAMA in 1999 by Schulz and Beach, followed spousal caregivers across four years and documented a 63 percent higher mortality rate among caregivers experiencing caregiver strain compared to non-caregiving controls, after adjustment for sociodemographic factors, prevalent disease, and subclinical cardiovascular disease. The relative risk figure (1.63) has been replicated across subsequent caregiver health research in multiple jurisdictions. Subsequent studies have documented elevated rates of cardiovascular disease, depression, anxiety, immune dysfunction, sleep disorders, and premature mortality among long-term caregivers compared to non-caregiving controls. The research base is substantial; the policy response is not.

The Canadian Centre for Caregiving Excellence's 2023 National Caregiving Survey and the 2024 follow-up data document the lived experience of the same patterns: caregivers report worse self-rated health than non-caregivers, higher rates of chronic conditions, reduced access to their own healthcare, deferred medical appointments, deteriorated nutrition, and elevated anxiety and depression. The 2018

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Statistics Canada General Social Survey documented that caregivers with the highest care intensity report the highest rates of negative health impact. The pattern is consistent: the more the family is carrying, the worse the family's own health becomes, and the more rapidly the family's own demand on the formal healthcare system increases.

The implication for Alberta's disability architecture is direct. Every dollar of public expenditure not made on PDD, AISH, FSCD, healthcare, and housing is recovered, in the short term, as private cost borne by family caregivers. The cost is then re-incurred by the same provincial budget, in the medium term, when the caregivers themselves develop the chronic illness, depression, and premature mortality the literature documents. The savings are illusory. The cost transfer is real. The harm is borne disproportionately by the families with the fewest resources to absorb it. The cumulative pattern is what produces the intergenerational disability concentration documented later in this section.

The Long Arc: Caregiving Across Decades

For families raising a child with a developmental, intellectual, or significant physical disability, the caregiving role does not end at age eighteen. It does not end at age twenty-five, or thirty-five, or fifty-five. The 2018 systematic review by Walker and Hutchinson, published in the *Journal of Intellectual and Developmental Disability*, examined fourteen studies of older parents of adult children with intellectual disabilities living at home or in the community. The review documented what families of children with intellectual disability already know: the caregiving role continues for the duration of the parent's life, and frequently for the duration of the disabled child's life as well.

Life expectancy for people with intellectual disabilities has improved substantially over the past several decades. More than forty percent of adults with Down syndrome now live to age sixty. Adults with autism, FASD, cerebral palsy, and other developmental conditions are living longer than in any prior generation, due to medical advances and improved access to care. The improved life expectancy is, on its own terms, an unambiguous good. It also creates a structural reality the public architecture has not adapted to: the caregiver is, in many cases, also aging, and the family is in an inverted situation in which both the disabled adult and the parent who has cared for them are now in need of formal supports the system has not built.

Walker and Hutchinson's review documents the concerns of older parents of adult children with intellectual disabilities directly. The dominant theme across the studies is anticipatory anxiety about the future. Parents are aware that they will eventually be unable to provide the care they have been providing. They are aware that their adult child will need an alternative. They are also aware that the alternatives the public system offers — supervised housing, group homes, day programs, residential placements through PDD — have multi-year waitlists, inadequate capacity, and uneven quality. The review documents a combination of factors that prevent older parents from completing future-care planning: lack of trust in existing services, lack of information about care options, mutual dependence between parent and adult child built over decades of shared life, and feelings of guilt or grief about the conversations themselves.

The phrase that recurs across qualitative research with these families is what aging parents of adult disabled children describe as *the question that does not have an answer*: what happens when we die. It is the question Inclusion Alberta's PDD survey respondents described in different words when they reported, in 2024, that adults with intellectual disabilities are being denied PDD support until their circumstances

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reach the point of crisis. For many families, the death of the primary caregiver is the crisis the urgent-and-critical-needs threshold is waiting for. The threshold is, in operational effect, a queueing mechanism that admits families to PDD-funded support at the moment the family safety net fails.

The cost is documented. The harm is documented. The province's response is to fund the program at a level below population growth and inflation, defund the advocacy organizations that document the pattern, and continue operating the urgent-and-critical-needs threshold as if the families' anticipatory anxiety were not, itself, a documented form of unmet need.

What Happens When the Caregiver Dies

The disabled adult whose primary caregiver dies enters a system that, in Alberta's current configuration, is not designed to catch them. The PDD waitlist does not move because a parent has died. The Outcome Plan that was supposed to have been developed in advance, per Inclusion Alberta's documentation, has not been developed in seventy percent of cases. The supervised housing placement that the family had assumed would be available is, in operation, not available. The group home that was on a regional waiting list has the same waiting list it had before the death. The disabled adult, frequently in their forties or fifties or sixties, is now navigating bereavement in addition to the loss of the practical infrastructure of their life.

The pathway that follows is documented across the disability literature in Canada and internationally. Some adults are absorbed by extended family — siblings, adult nieces or nephews, in some cases adult children of siblings — who take on the caregiving role the deceased parent was performing. The extended family then enters the same caregiver burden cycle, frequently with less preparation and at a later life stage. Some adults enter emergency PDD-funded placements that the system finds, at higher cost than planned placements, because the urgent-and-critical-needs threshold has now been met. Some adults enter long-term care facilities designed for elderly residents, which are clinically inappropriate for adults with developmental disabilities but which have available beds. Some adults enter homelessness. Some adults enter the justice system, frequently for behaviours that the absence of structured support produces. Some adults enter hospitals, where they remain as bed-blockers because no community placement is available to discharge them to. Some adults die.

None of these pathways is a placement. All of them are absorptions of the public-system failure into systems designed for different purposes, at multiples of the cost that proper PDD architecture would have required. The pattern is documented. The province does not publish data on the population of disabled adults whose primary caregiver has died and who have not been placed in PDD-funded support. Inclusion Alberta has, repeatedly, requested that data. The Government of Alberta has not, in three years, released it. The absence of the data is itself a form of policy — the population whose existence is not measured does not appear in budget projections, program design documents, or political accountability.

The structural argument is the same as the structural argument across Sections 1.3 and 1.4. The system has approved the population for the program. The program does not, in operation, deliver the support the approval implies. The cost is borne by the family until the family fails. The cost is then borne by the population — in homelessness, hospitalization, institutionalization, or premature death — that the program was supposed to support.

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The blueprint Part Three builds will require, at minimum, a binding statutory provision that no PDD-eligible adult shall be without a placement plan that is current, deliverable, and known to the family at the moment of the primary caregiver's death. The legislation Part Three proposes is the architecture that makes that provision enforceable. The argument Part Four makes is that the absence of such a provision in current Alberta law, despite decades of family advocacy requesting it, is a choice.

Generational Disability

The lifecycle failure pattern documented across Sections 1.3 through 1.5 does not stop at the family in front of it. It produces the next family the same architecture will fail. The mechanism is partly biological and partly social. Both are documented. Together they explain why disability concentrates in particular families across generations, and why those generations, in aggregate, bear a disproportionate share of the cost of the system's underbuilding.

The biological mechanism is straightforward. Many disabilities have heritable components. Autism, ADHD, intellectual disability, FASD risk in subsequent pregnancies, certain chromosomal conditions, certain neurological and psychiatric conditions, certain physical disabilities — these and others have documented genetic, epigenetic, or familial-environmental components that produce above-baseline rates of recurrence within families. A family with one disabled member is statistically more likely than a randomly-selected family to have additional disabled members across generations. This is not stigma. It is population genetics applied honestly. It does not mean disability is a family failing. It means disability tends to cluster in families for reasons the family did not choose.

The social mechanism is the part the lifecycle frame makes visible. Children growing up with insufficient developmental support do not become adults with no consequences. They become adults with foreclosed capacity, foreclosed opportunity, and foreclosed earning potential, as documented in the chronological evidence chain in Section 1.3. Those adults have children. The children grow up in households shaped by the unsupported disability of the parent — in poverty, in housing instability, in food insecurity, in the chronic stress that adverse childhood experiences research has consistently linked to elevated risk of subsequent developmental, behavioural, and health outcomes for the children themselves.

The Alberta ACE Study, which surveyed Alberta adults on their childhood adversity exposure, documented that 55.8 percent of Albertans reported one or more adverse childhood experiences. Twenty percent reported three or more. The research base on adverse childhood experiences, beginning with Felitti and colleagues' 1998 study and replicated extensively across the subsequent two decades, has established that ACEs predict elevated risk across virtually every adult health, mental health, educational, employment, and longevity outcome studied. One frequently-cited estimate, summarized in Public Health Ontario's *Adverse Childhood Experiences: Interventions to Prevent and Mitigate the Impact of ACEs in Canada* (2020), is that exposure to six or more ACEs may decrease lifespan by twenty years. Another, from the same source, is that a ten-percent reduction in ACE prevalence in North America would yield a corresponding reduction of one million disability-adjusted life-years — approximately US\$56 billion in averted cost.

The populations bearing the highest concentrations of adverse childhood experiences in Canada are well-documented. Indigenous communities carry elevated ACE prevalence as a direct documented

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consequence of intergenerational trauma from residential schools, child welfare apprehensions, and the broader policy architecture of colonization. Low-income families across populations carry elevated ACE prevalence. Families with parental mental illness, substance use, or imprisonment carry elevated ACE prevalence. Families with previously unsupported disability carry elevated ACE prevalence. The categories overlap, and the overlap is the population the public disability architecture is, in operation, building toward by under-building support upstream.

The campaign's separate 195-page Indigenous Disability evidentiary record, referenced elsewhere in the document library, examines the Indigenous-specific dimensions of this pattern in detail. The companion *Intergenerational Family Report* and *Intergenerational Contradiction Report* document the family-level patterns across the broader Alberta disabled population. Section 1.5 names the principle the more detailed records examine: the lifecycle failures documented across Part One produce the next generation of population that the same system will fail in the same ways. The pattern is intergenerational because the failure is intergenerational.

A province that does not build proper growth-mode architecture in childhood produces adults with foreclosed capacity. Adults with foreclosed capacity, raising children in poverty, produce children with elevated ACE exposure. Children with elevated ACE exposure produce adults with elevated rates of disability, mental illness, addiction, and premature mortality. Those adults raise the next generation. The cycle is the architecture's output. The architecture continues to operate as if the cycle were the population's pre-existing condition rather than the architecture's documented product.

What the Aging Parent Sees

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ILLUSTRATION — A PARENT AT SEVENTY-TWO

Imagine a parent who is now seventy-two. Their adult child, now forty-six, is autistic with a co-occurring intellectual disability. The child has lived at home for the entire forty-six years. The child is on the PDD waitlist. The child has been on the PDD waitlist for fourteen years, since the moment the family applied at the child's thirty-second birthday after the parent's first cardiac event made the future-care question urgent. The child's name has not moved measurably up the waitlist in those fourteen years. The Outcome Plan that the family was told would be developed has, per the pattern Inclusion Alberta has documented, not been developed. The family is unaware that this is the case, because the family has not heard from a caseworker.

The parent is seventy-two and has had two further cardiac events since the application was filed. The parent's spouse, who shared the caregiving role, died eight years ago. The parent's siblings are themselves elderly. The parent's other adult child, the disabled adult's sibling, lives in another province and has young children of their own and a job and a life that does not include the kind of full-time caregiving the disabled adult requires. The parent has had the conversation with the sibling about what happens when the parent is gone. The conversation did not produce an answer. It produced a series of contingency outlines that depend on the public system providing what the public system has not, in fourteen years, provided.

The parent does the math the parent has been doing for two decades. The mortality tables suggest the parent has between five and fifteen years remaining. The PDD waitlist has not moved meaningfully in fourteen. The supervised housing placements that exist in the parent's region are full and have multi-year waiting lists of their own. The group homes that the parent has visited are clinically appropriate for some of the residents and clinically inappropriate for others; the parent has no confidence that the placement the disabled adult would receive in the moment of crisis would be appropriate to the disabled adult's specific needs.

The parent has written a letter that the disabled adult's sibling will receive when the parent dies. The letter contains the names of three case managers, the case file numbers from PDD and AISH, the names of two physicians, the address of the family doctor of fifteen years (now retired), the address of the walk-in clinic, the bank account numbers, the medication list, the dietary preferences, the sensory requirements, the routines that have organized the disabled adult's life for forty-six years, and the parent's request that the sibling do everything possible to prevent the disabled adult from being placed in a long-term care facility designed for elderly residents.

The letter cannot guarantee the request. The letter is what the parent has, in the absence of the architecture the public system was supposed to have built. The parent has been carrying that absence for forty-six years. The parent is seventy-two. The parent will not be carrying it indefinitely.

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The disabled adult does not know any of this. What the disabled adult knows is the daily routine of the home the family built, the rhythms the parent has established over forty-six years, the meals at the times they happen, the weekly outing to the same coffee shop on the same morning, and the specific way the parent says good morning. None of this is in the case file. None of this is in the PDD database. None of this transfers to the placement the public system might or might not provide on the day the parent dies.

What This Section Establishes

The family is what the system depends on. The family is also what the system is consuming. The economic value of unpaid family caregiving in Canada, between \$97 billion and \$112.7 billion annually, is the measure of what the formal system does not provide. The health cost to caregivers, documented through elevated mortality, morbidity, and chronic disease across decades of peer-reviewed research, is the cost of carrying what the formal system has transferred to the family. The aging-parent question — what happens when we die — is the question the urgent-and-critical-needs threshold is, in operational effect, waiting to answer through the death itself.

The intergenerational pattern is the architecture's documented output, not the population's pre-existing condition. Adverse childhood experiences research, the Alberta ACE Study, the international developmental literature, and the Canadian Indigenous-specific evidence all converge on the same finding: under-building support for disabled children and disabled adults produces the next generation's disabled population. The cycle is the architecture's product. The architecture continues operating as if the cycle were a separate problem.

The blueprint Part Three builds will require, at minimum, the following caregiver-related commitments: statutory recognition that family caregivers are providing publicly necessary work, with associated compensation through Family Managed Services or equivalent mechanisms; binding placement-plan provisions for PDD-eligible adults with aging primary caregivers, current as of each anniversary review; restoration of the advocacy organizations that have been documenting the family safety net's exhaustion; and integrated transition planning that begins not at the disabled adult's eighteenth birthday but at the primary caregiver's sixty-fifth, on the recognition that the caregiver's mortality is the architectural feature the public system has been ignoring. The legislation Part Three proposes is the architecture that makes those commitments enforceable. The argument Part Four makes is that the under-building of caregiver support, despite decades of family advocacy requesting it, is a choice.

Section 1.6 closes Part One by examining the disability category whose entry into the system produces the most coherent triage response Alberta has built — acquired disability through workplace injury — and the comparator question that follows. Why does the province treat the disability that arrives through one route as a triage emergency, with full rehabilitation, full income protection, full continuity of medical care, and no requirement that the injured worker re-prove the injury every six months — while treating the disability that arrives through any other route as a population to be tested, doubted, narrowed, and reassessed? The answer is documented in the architecture. Section 1.6 examines what the architecture's

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selectivity reveals.

The family safety net has been holding the line for the populations the system has not built for. The line is not infinitely holdable. Section 1.6 closes Part One on the question the rest of the document is built to answer: what would it look like if the province built the architecture properly, for everyone?